

11.5% One in eight first-born babies in the US are born using assisted delivery.

Q Who will perform the assisted delivery if it becomes necessary?

The procedure will be done by your own doctor, possibly with help from a nurse who has had specialized training in assisted deliveries. During the procedure, the delivery room may feel quite busy since you will have your doctor and some nurses present, plus a pediatrician, who will be there just in case your baby needs attention after the birth. The presence of the pediatrician is often precautionary, but if you're concerned, ask your doctor to explain everyone's role, which he or she will be happy to do.

Q Which is better, forceps or vacuum extractor, and will I have a choice?

Both types of delivery are considered safe and effective, but the choice of which type depends on the medical circumstances; thus it is best to be guided by your doctor. For example, vacuum extractor is the preferred method if your baby needs to rotate to move down the birth canal. This method is less likely to cause injury to your vagina and/or perineum. On the other hand, forceps are gentler for the baby, but can increase the risk of trauma to you. But if your baby is in difficulty, your doctor's priority is to act quickly and forceps will provide the faster delivery. If your doctor begins with the vacuum extractor and the attempt is unsuccessful, he or she may decide to try again using forceps. However, if a forceps delivery is unsuccessful, whether this is used first or after a vacuum extractor attempt, you will usually need to have a cesarean section.

Q How long will the assisted delivery procedure take?

Once you have been prepared for the procedure and the forceps or vacuum extractor is attached to your baby's head, you will usually be guided through a maximum of three pushes to deliver.

If your baby isn't born after three pushes, your doctor will decide whether to try again, to switch to a forceps delivery (if you started with vacuum extractor), or to send you for an emergency cesarean section. Your doctor will be with you the whole time, explaining what is happening at each step. If you are having an assisted delivery because an epidural has made it hard for your baby to rotate, your doctor will tell you exactly when to push to help deliver the baby.

Q Are there risks for me if I have an assisted delivery?

You may have some vaginal or anal tearing, or need an episiotomy, which will be stitched after the birth. If there is bleeding during delivery there will be an increased risk of developing a

blood clot (as with any birth where bleeding occurs). Approximately a third of women who have had an assisted delivery have some urinary incontinence afterward, so talk to your doctor about Kegel exercises to help speed your recovery. Occasionally, there may be trauma to the anal sphincter (the circular muscle around your anus), which can lead to temporary fecal incontinence.

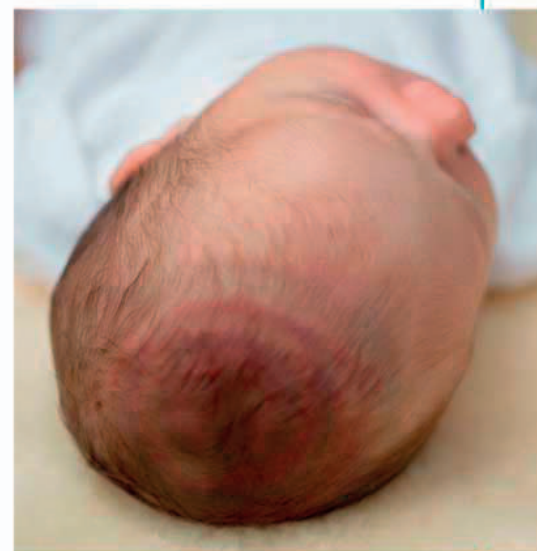
Q How can I avoid an assisted delivery in the first place?

Being well-supported throughout labor, by both a doctor and a birth partner who remains with you, can help you to avoid an assisted delivery. Staying upright during labor or lying on your side can help with your baby's position and descent in the birth canal, reducing the need for an assisted delivery. Stay calm and be guided by your doctor about when to push to help prevent you from becoming exhausted. Avoiding an epidural also reduces the chances that you will need help with the delivery.

Q How will my baby be affected by an assisted delivery?

An assisted delivery often leaves a mark on your baby. Swelling, bruising, and cuts are all common, but these effects soon disappear after the birth.

It's common for a baby's head to look elongated after a vacuum extractor delivery due to the effects of the suction cup. Referred to as a "chignon," this swelling doesn't cause any damage to your baby's skull or brain, and your baby's head will return to its regular shape within 24–48 hours of the birth. Your baby may also have a small bruise, called a cephalic hematoma, where the suction was strongest, which again should disappear quickly. The stress of a vacuum extractor birth is thought to be the reason that vacuum extractor babies have lower than average apgar scores (see p.238) at five minutes after birth; however, these return to normal by 10 minutes. After a forceps birth, it's common for babies to have small marks on their face, and there may be cuts on the face and head, which heal and disappear quickly after the birth.



Marks and bruises These are common after an assisted delivery, but these side effects are superficial and will soon disappear.